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Name: عبير فهمى شلبى الشربيني National ID: 27505031601629	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

Patient Name: Abeer Fahmy Shalabi Al Sherblny	Date : 19/07/2025
Exam : Item 1	Age : 50 Year(s)
Medical ID:	Pat. ID: 43314

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

I Chest:-

- Known case of breast cancer showed:
- Left upper lobar anterior segment subpleural reticulations with few atelactatic bands likely post therapeutic changes.
- Otherwise, Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size with homogenous CT texture. No definite enhancing focal lesions or contour irregularities.
- No Intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:** no definite worrisome lytic or sclerotic focal lesions.

Opinion:

- Left upper lung lobe anterior segment subpleural reticulations with few atelectatic bands likely post therapeutic changes.
- No evidence of pulmonary, hepatic or osseous metastatic deposits.

Signature Dr.Mohamed Hassan
23/07/2025 04:07



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



BRCA 2 +ve

Patient ID:

Name: محمد فهمي شبيب الشرباش

Sex:

MDT form

3- Medical numbers:
Female

83172118

Old Med:

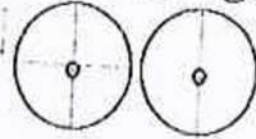
Old Med

4234/2023

2-ID number: 27505031601629



Patient Age: 49 Y



Best-NAC
clinical

Family history

Family history
+VE

Phone

Phone
01065810262

BMI

BMI
29

Marital status

Marital status
Married

NRS Score

NRS Score

GROUP

GROUP
A

REASON OF MDT

REASON OF MDT
POST NAC

Co morbidity

Co morbidity
HTN

Menopausal status

Menopausal status
Menopausal

Initial STAGE

Initial STAGE

Date of MDT

Date of MDT

15/05/2024

Breast Imaging:

Lt breast 3 o'clock BIRADS 4b

Clinical data

Mets Workup:
CT CAP: free

DX	ER	PR	Her 2 Neu	SISH	Ki67
ER COMMENT -VE	ER COMMENT -VE	PR COMMENT -VE	Her 2 Neu COMMENT -VE	SISH COMMENT	Ki67 80%
T 2	N 0	M 0			

Sonogramography

PATIENT HISTORY

- 1-CESM: Re MDT after CESM (if unifocal for clip insertion)
 - 2- ECHO
 - 3- Labs
 - 4- BRCA testing
- Pt will be for NACT (AC* 4 then taxol/Carb*12)

MDT OUTCOME & PLAN

for wire localization the WLE & SLNB (central quadrenectomy)
Pt is BRCA 2 mutant

SIGNATURE			
MEDICAL ONCOLOGY	RADIOLOGY		
SURGERY	PATHOLOGY		
Psychiatric	Nutrition		

Patient Name : الأمينة / عزيزة محمد شادي الشربيني
Patient No. : 27505031601629
Path No. : 1745/24
Sex : Female
Age : 49 Y
Referring Doc :

Request No. : 13001202
Request Date : 26/05/2024
Verification Date : 02/06/2024
Financial Category : اورام نادر السلام - مبادره
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer, post NACT.

Nature of specimen: Left WLE & left SLNB

Gross examination:

• Procedure: Left BCS

Fibrofatty tissue piece measured 15x8x6 cm with covering skin ellipse measured 12x4 cm.

• Tumor extent : A greyish white tumour bed site

• Size: 5x3x2 cm.

• Skin involvement: Unremarkable grossly.

• Distance from margins: 1 cm away from least deep margin

• Frozen section: Free margins & free SLNB (0/4)

Microscopic examination:

• Histological type: No residual tumor tissue

• Ductal carcinoma in situ: No

• Lymph nodes: Free (0/4)

• Skin involvement: Free

• Resection margins: Free margins

• Histologic response to therapy: Pathologic complete response

Dignosis:

Left WLE & Left SLNB (post NACT) :

No residual tumor tissue

• Free margins

• Free lymph nodes (0/4)

• Pathologic complete response (PPR)

• Stage : ypT0, N0

Dr. Rana Fathy
Dr.Heba Hassan

Verified by :
Dr.Heba Hasan

Printing Date : 22/06/2024 08:56:55

Page 1 of 2

Printing By : Mrs.Mayada Hegazy

CAP

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - نيلز النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Miss El Qadema - Cairo, Egypt

Patient Name: Abeer Fahmy Shalabi Al Sherbiny	Date : 24/10/2023
Exam : MAMMOGRAPHY BOTH	Age : 48 Year(s)
Medical ID: 27505031601629	Pat. ID: 43314

**DIGITAL BILATERAL SOFT TISSUE MAMMOGRAPHY AND
COMPLEMENTARY ULTRASOUND**

Indication:

- Mastalgia
- Positive family history (sister)

Report:

- Heterogeneous fibroglandular parenchyma (ACR c)
- Left Breast:
 - Ill-defined equally dense obscured lesion is seen in the para-areola region at 3O'clock extending retro-areola with suspected intra-duct extensions reaching the nipple. By ultrasound it is seen as an ill-defined hypoechoic irregular mass measuring 30x25x20mm about 8mm from the skin. Peripheral vascularity of equivocal RI = 0.6 is noted.
 - No pathological enlarged axillary, infra or supraclavicular lymph nodes.
- Right Breast:
 - No suspicious micro calcifications or architectural distortion detected.
 - Complementary ultrasound no cystic or solid mass lesions.
 - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
 - Nonspecific axillary L.N.

Opinion:

- Left breast suspicious mass likely neoplastic (BIRADS 4b) for histopathological examination.
- Normal sonomammography of right breast (BIRADS I)

Best regards,

Dr. Amira H Radwan MD,
Breast Imaging consultant,
NCL, Cairo University
24/10/2023 11:41 AM

Patient's Name: Abeer Fahmy Shalaby El Sherbini
Date: 2026-05-30
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

INDICATION:

Left CBS

TECHNIQUE:

- Bilateral MLO and CC views
- Bilateral U/S

REPORT:

- Dense parenchyma, (ACR C), with focal disruption seen underlying the scar site.
- No suspicious mass lesions, suspicious micro calcifications or architectural distortion detected.
- Complementary U/S showed no solid or cystic mass lesions.
- Normal skin thickness and nipple-areola complex.
- Right nonspecific axillary nodes are seen
- Free left axillary region as well as the infra and supra-clavicular regions.

OPINION:

- Normal study of the right breast, (BIRADS 1).
- Left post-operative changes,(BIRADS 2).

Best regards,
Dr. Yara Mohamed, MD